

# Coverage of obesity drugs in Medicaid: what it did, who it reaches, what it could cost

Insight deck · public aggregate data, 2018 to early 2026

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# The answer: coverage lifts prescriptions steadily, and a 1,000,000-enrollee programme would net about \$161.3 million over five years

**12.2**

extra prescriptions  
per 1,000 enrollees  
per quarter  
95% CI 8.4 to 16.0

**1.3 to 17.2**

per 1,000 enrollees  
per quarter, first  
quarter to eighth  
(market growth  
included)

**\$55.6 to  
369.4  
million**

five-year net cost  
across scenarios,  
1,000,000 enrollees

**\$2.69**

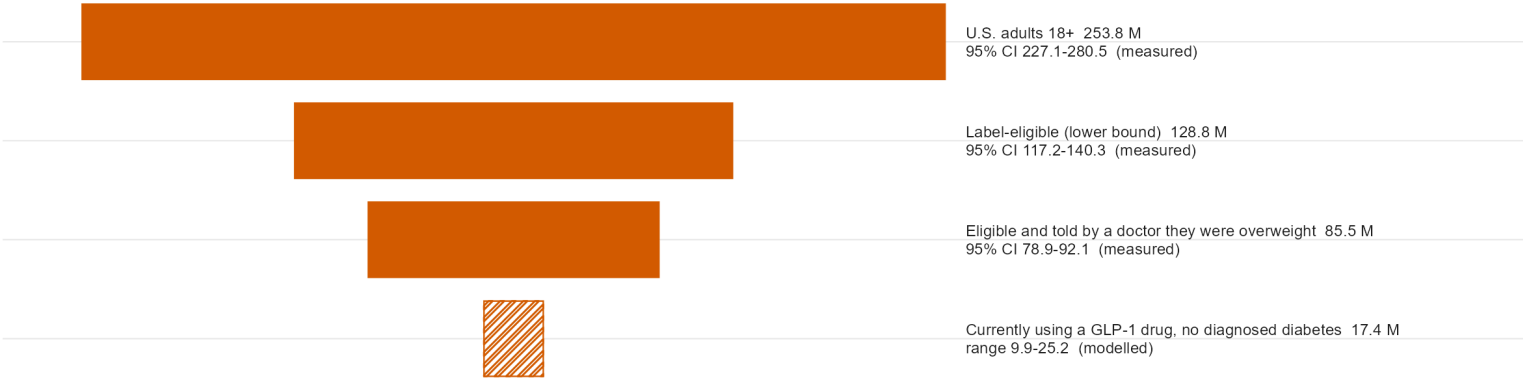
net per enrollee per  
month, central case  
(rebate 51.2% is  
assumed)

Sources: CMS State Drug Utilization Data and Medicaid enrollment (Module C); Module E budget model. Net of an assumed rebate; SDUD amounts are gross. Scenarios, not forecasts.

# Market today: about half of U.S. adults meet the label criteria, and about 30.4 million already use a GLP-1 drug

Solid bars: measured (NHANES 2021-2023 unless stated). Hatched bars: modelled from the KFF Health Tracking Poll (all GLP-1 drugs, all indications). The eligible count is a lower bound: sleep apnea and other label conditions are not measurable in NHANES.

## Obesity pathway



## Type 2 diabetes pathway

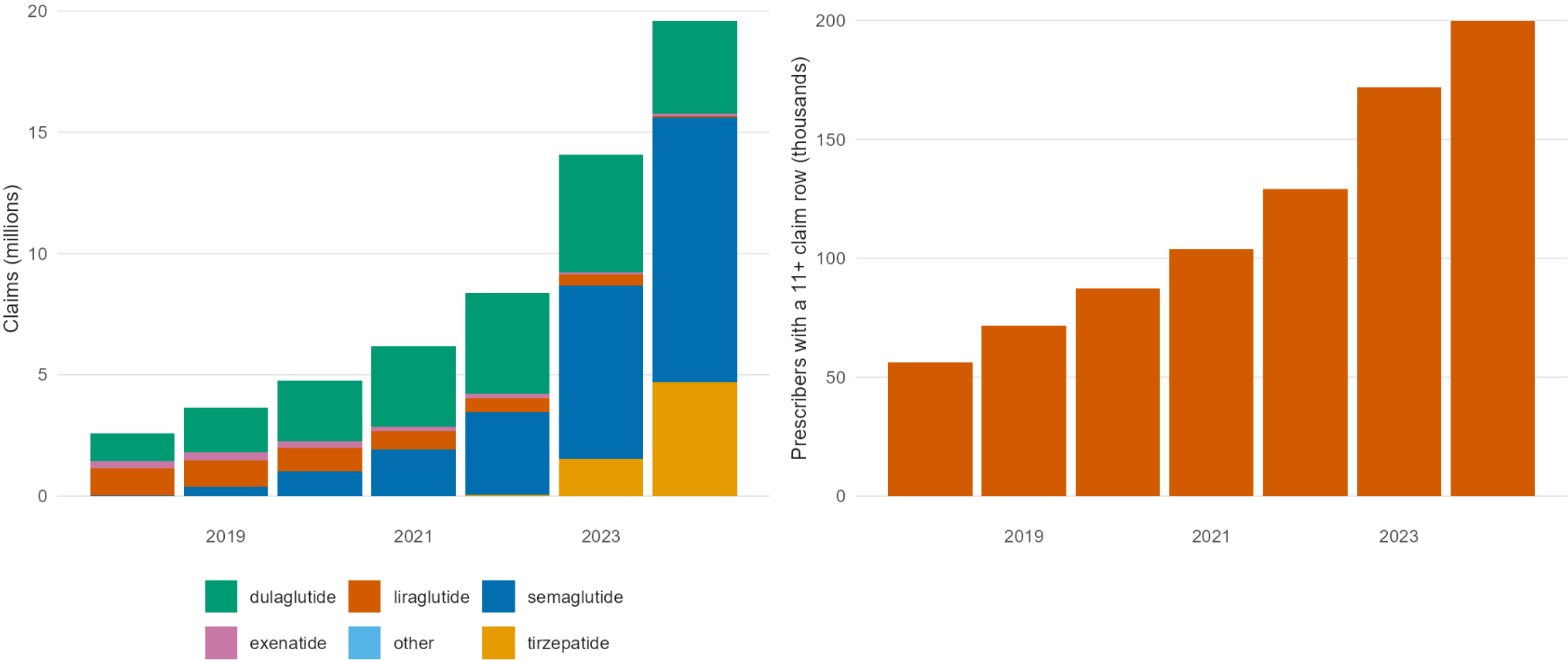


Sources: NCHS NHANES; KFF Health Tracking Polls (Feb 24-Mar 2, 2026 and Nov 2025); FDA Wegovy and Zepbound labels. Bar widths are proportional to people. Told-overweight step uses the 2017-2020 file (not asked in 2021-2023).

Source: NHANES 2021-2023 (measured) and KFF poll, February 24 to March 2, 2026 (modelled). Eligible is a lower bound: 128.8 million adults (95% CI 117.2 to 140.3 million).

# Market today: Part D incretin claims grew from 2.6 million to 19.6 million in six years, mostly for diabetes and other covered uses

Left: claims by ingredient. Right: distinct prescribers with at least one 11-claim row.

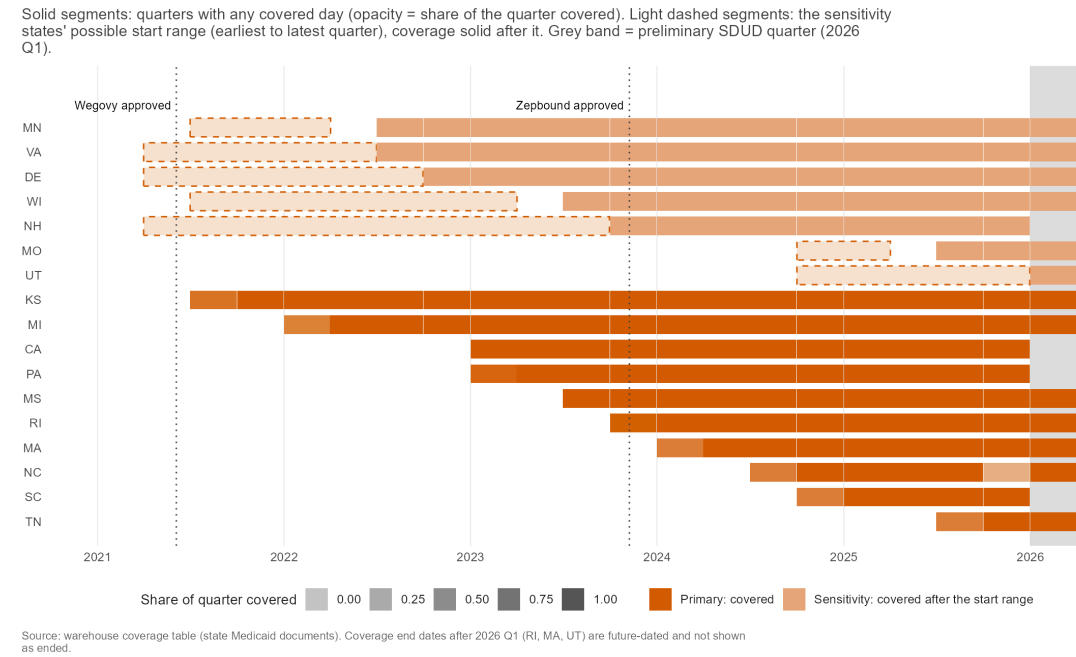


Source: CMS Medicare Part D Prescribers by Provider and Drug (rows with at least 11 claims), NPPES. Part D reflects diabetes and other covered uses, not obesity-brand adoption; counts are claims, and only prescribers with 11 or more claims for a drug appear.

Source: CMS Medicare Part D Prescribers by Provider and Drug (rows with at least 11 claims). Part D reflects diabetes and other covered uses, not obesity-brand adoption.

# Landscape: injectables, new oral options, a Medicare Bridge and a \$245 price are arriving while some states end coverage

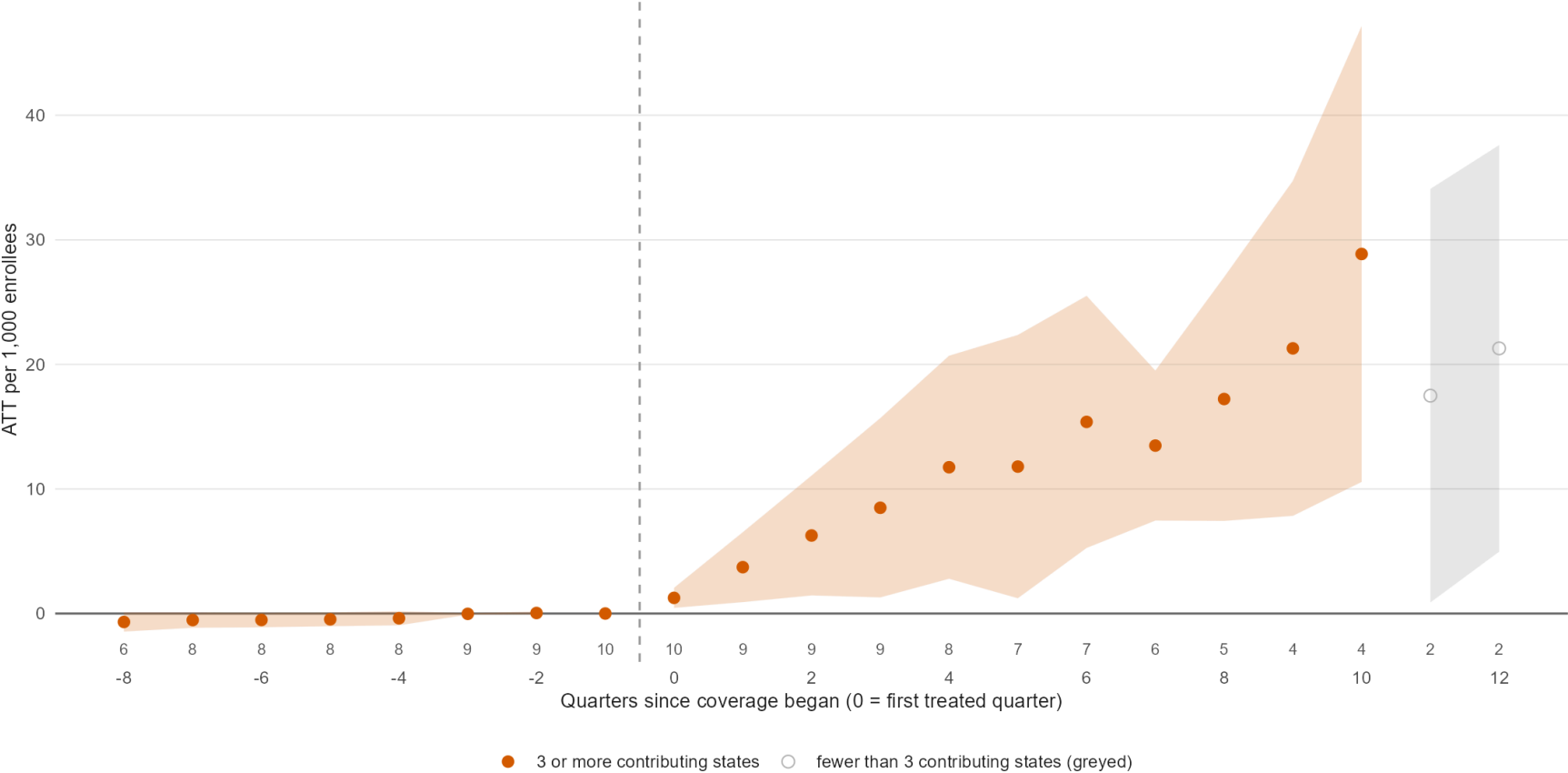
- **Products.** Wegovy (injection and tablets), Zepbound (KwikPen) and Foundayo (tablets) are covered under the Medicare GLP-1 Bridge (CMS prescriber page, June 2026).
- **Medicare GLP-1 Bridge.** A short-term demonstration, July 1, 2026 to December 31, 2027, with a \$50 copay (CMS).
- **Price.** The White House (November 2025) announced a Medicare price of \$245 a month for these drugs and said state Medicaid programmes will have access at those prices.
- **State coverage is changing** (timeline below): several states ended or paused coverage in late 2025 and early 2026.



Sources: CMS Medicare GLP-1 Bridge pages (saved under docs/sources); White House fact sheet, November 2025; state Medicaid documents (data/reference/medicaid\_obesity\_coverage.csv).

# Access: coverage added 12.2 prescriptions per 1,000 enrollees per quarter, rising from 1.3 to 17.2 over eight quarters

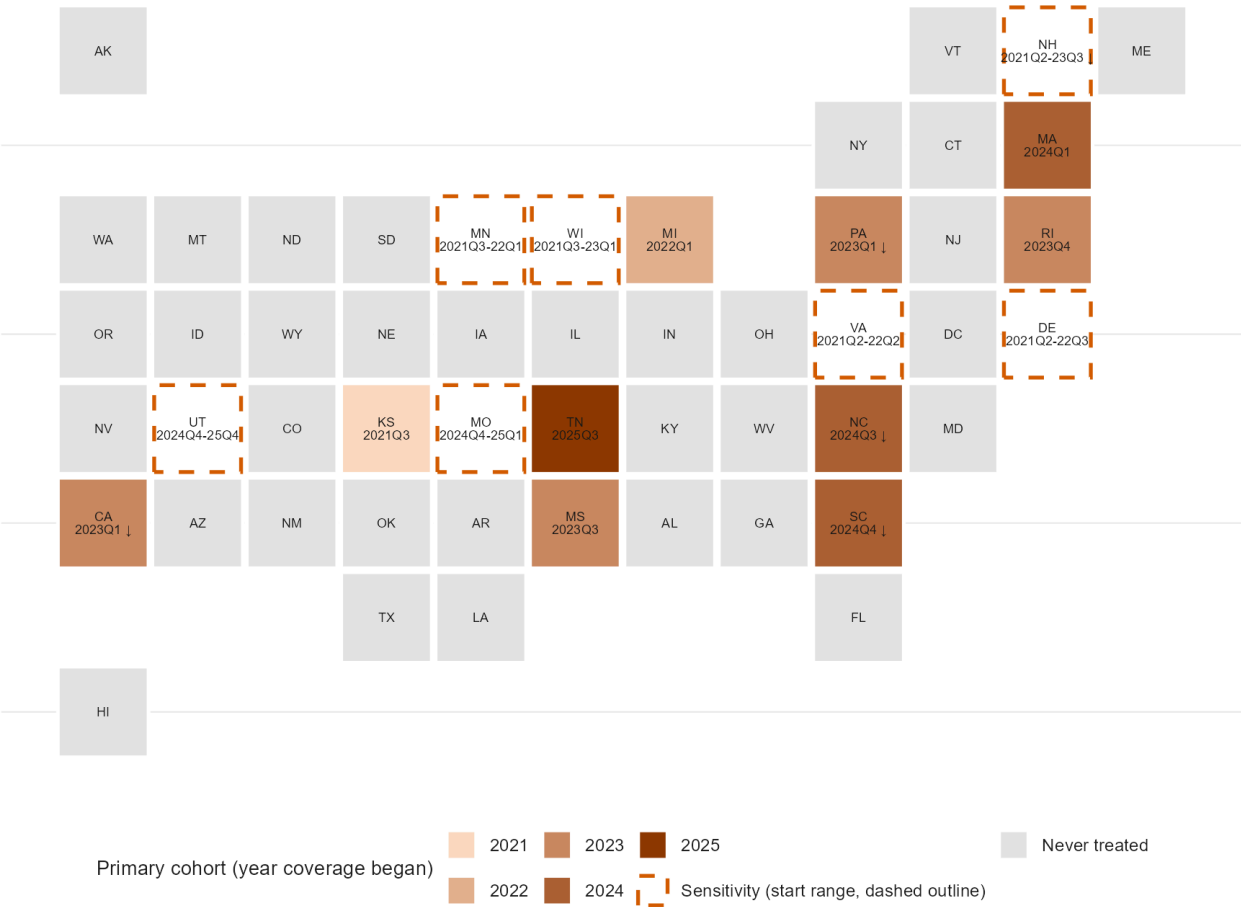
Callaway-Sant'Anna dynamic ATT, 10 primary states vs never- and not-yet-treated states, 2018 Q1 to 2025 Q3; 20 imputations of suppressed cells combined with Rubin's rules. Shaded band and points: pointwise 95% CI; numbers along the bottom: contributing treated states; event times with fewer than 3 contributing states are greyed.



Simultaneous confidence bands are not shown: with single-state cohorts the bootstrap critical value is unreliable (did warns of this) and very large (17.9 on average over the imputations, versus 1.96 pointwise). Leads before 2021 Q2 (before Wegovy existed) are omitted: the outcome is mechanically zero there and they are not evidence of parallel trends. Source: CMS State Drug Utilization Data and Medicaid enrollment (warehouse marts). Gross of rebates; counts under 11 suppressed by CMS.

# Access: ten states drive the estimate, and coverage dates differ by state

50 states and DC: 10 primary (cohort = first quarter with coverage), 7 sensitivity (earliest-latest possible start quarter shown), 34 never treated. ↓ = coverage later ended or lapsed.

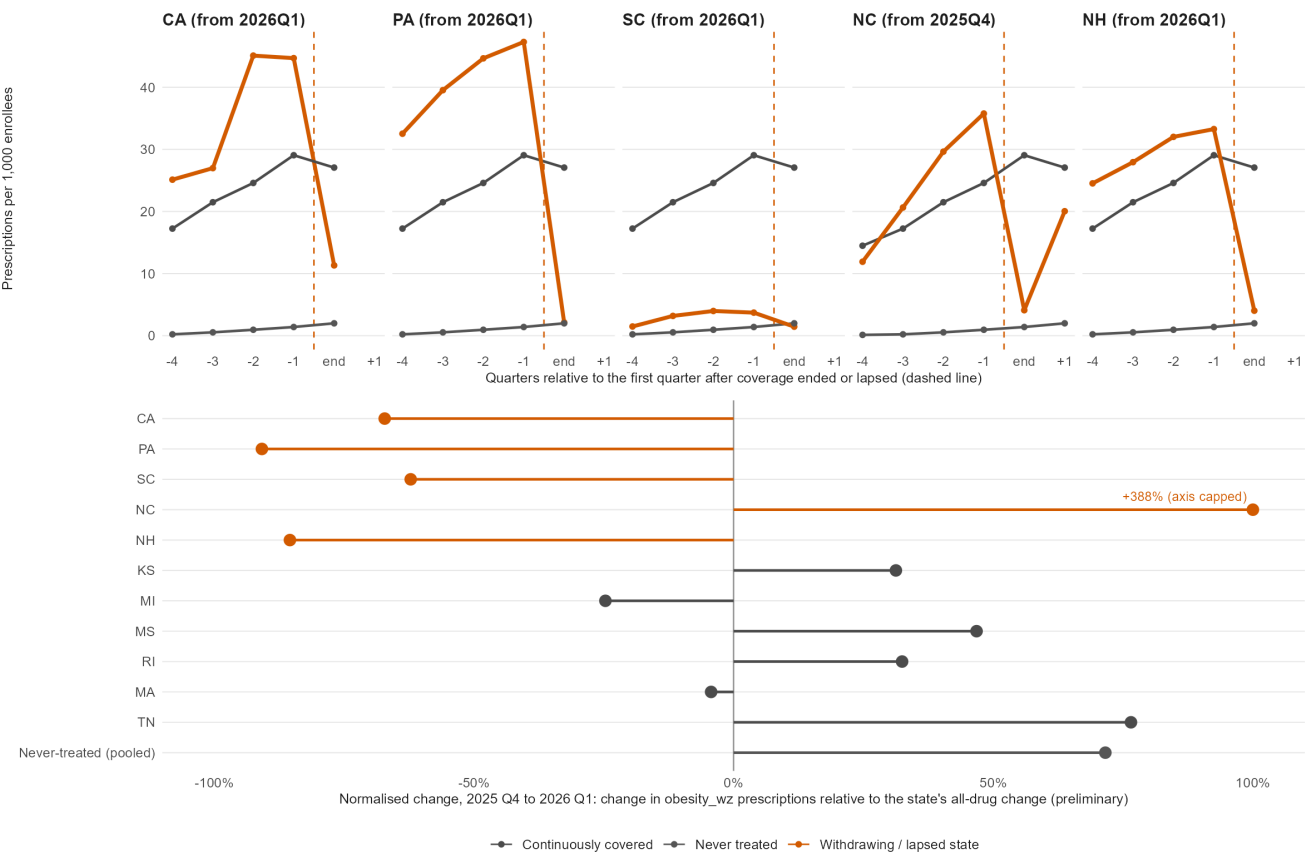


Source: state Medicaid documents compiled in the warehouse coverage table (medicaid\_obesity\_coverage). Data through SDUD 2026 Q1 (preliminary).

Source: state Medicaid documents and CMS approvals (docs/sources). 9 of 10 covering states recorded prior authorization. Seven states with uncertain start quarters are used only in a sensitivity analysis.

# Access, detail: coverage operates through managed care (South Carolina, Rhode Island), and the first withdrawals show how fast it can reverse

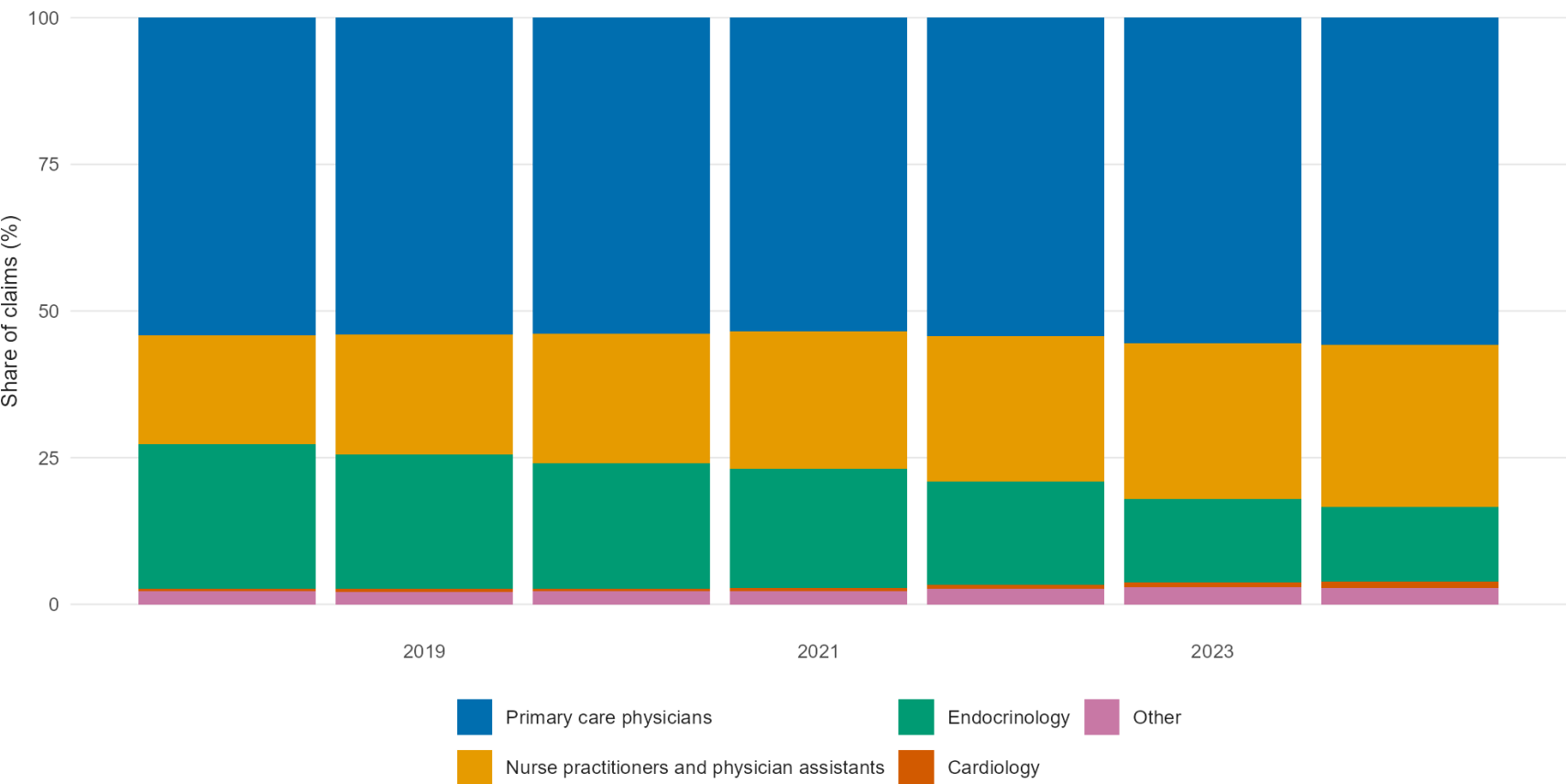
Descriptive only; no model, no p-values; 2026 Q1 is preliminary SDUD data. Normalised change =  $(1 + \text{obesity\_wz change}) / (1 + \text{all-drug SDUD change}) - 1$  for 2025 Q4 to 2026 Q1; raw changes are in the table (obesity\_wz: CA -75.7%, PA -95.5%, continuously covered median -1.6%; all drugs: CA -26.0%, PA -51.0%, median -17.9%).



Source: CMS State Drug Utilization Data and Medicaid enrollment (warehouse marts). Gross of rebates; counts under 11 suppressed by CMS. PRELIMINARY: 2026 Q1 is preliminary SDUD data; only one post-withdrawal quarter exists for most states (North Carolina lapsed 2025-10-01 to 2025-12-11 and resumed). New Hampshire is a sensitivity state.

# Prescribers: primary care and nurse practitioners and physician assistants wrote 55.8% and 27.7% of Part D incretin claims in 2024

Share of Part D incretin claims by prescriber specialty group, 2018-2024 (CMS prescriber type grouped by the committed specialty map)

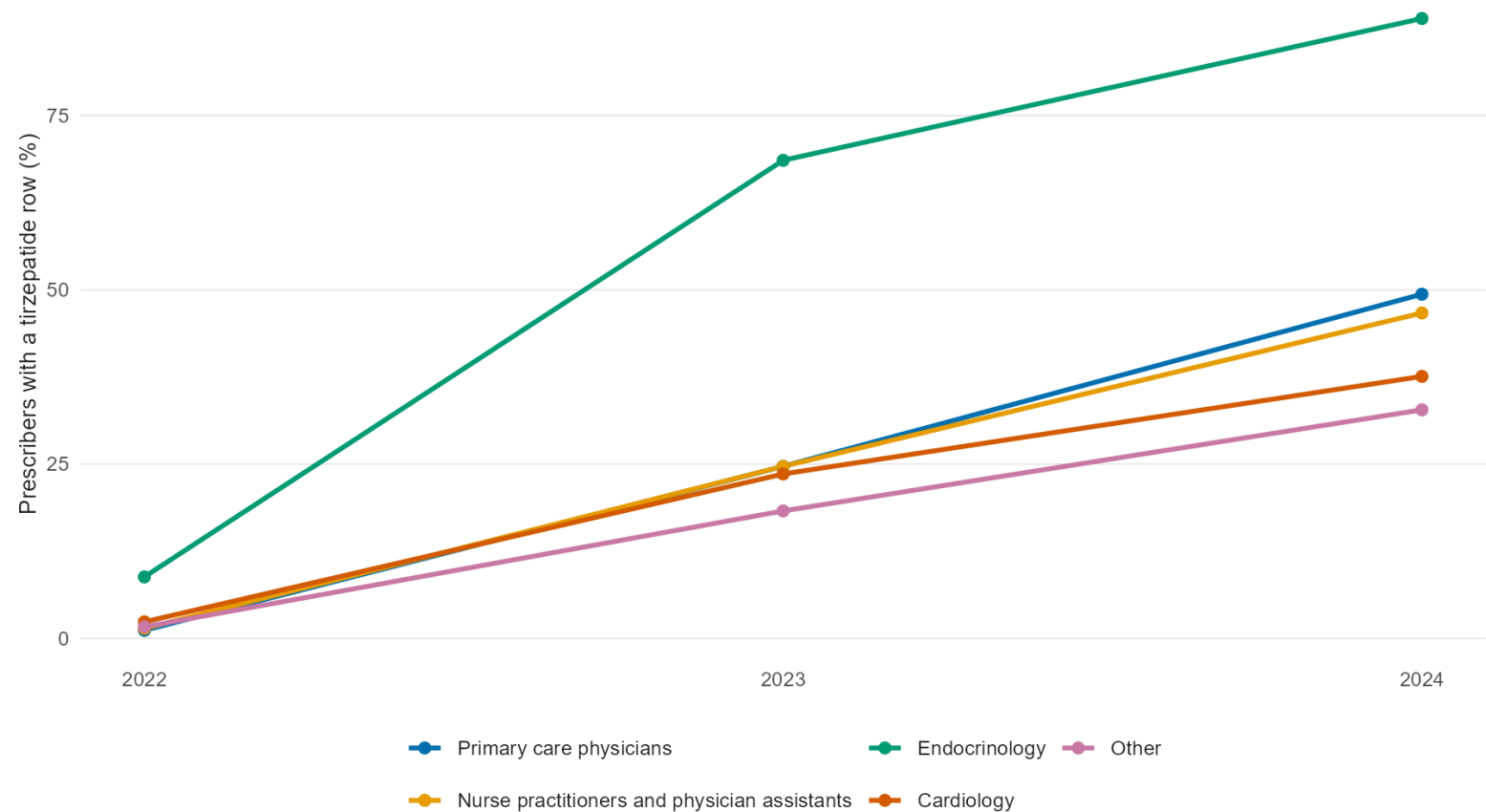


Source: CMS Medicare Part D Prescribers by Provider and Drug (rows with at least 11 claims), NPPES. Part D reflects diabetes and other covered uses, not obesity-brand adoption; counts are claims, and only prescribers with 11 or more claims for a drug appear.

Source: CMS Medicare Part D Prescribers. Part D reflects diabetes and other covered uses, not obesity-brand adoption.

# Prescribers: tirzepatide reached 48.6% of Part D incretin prescribers by 2024, from 1.6% in 2022

Share of prescribers with at least one incretin row that have any tirzepatide (Mounjaro) row, by specialty group

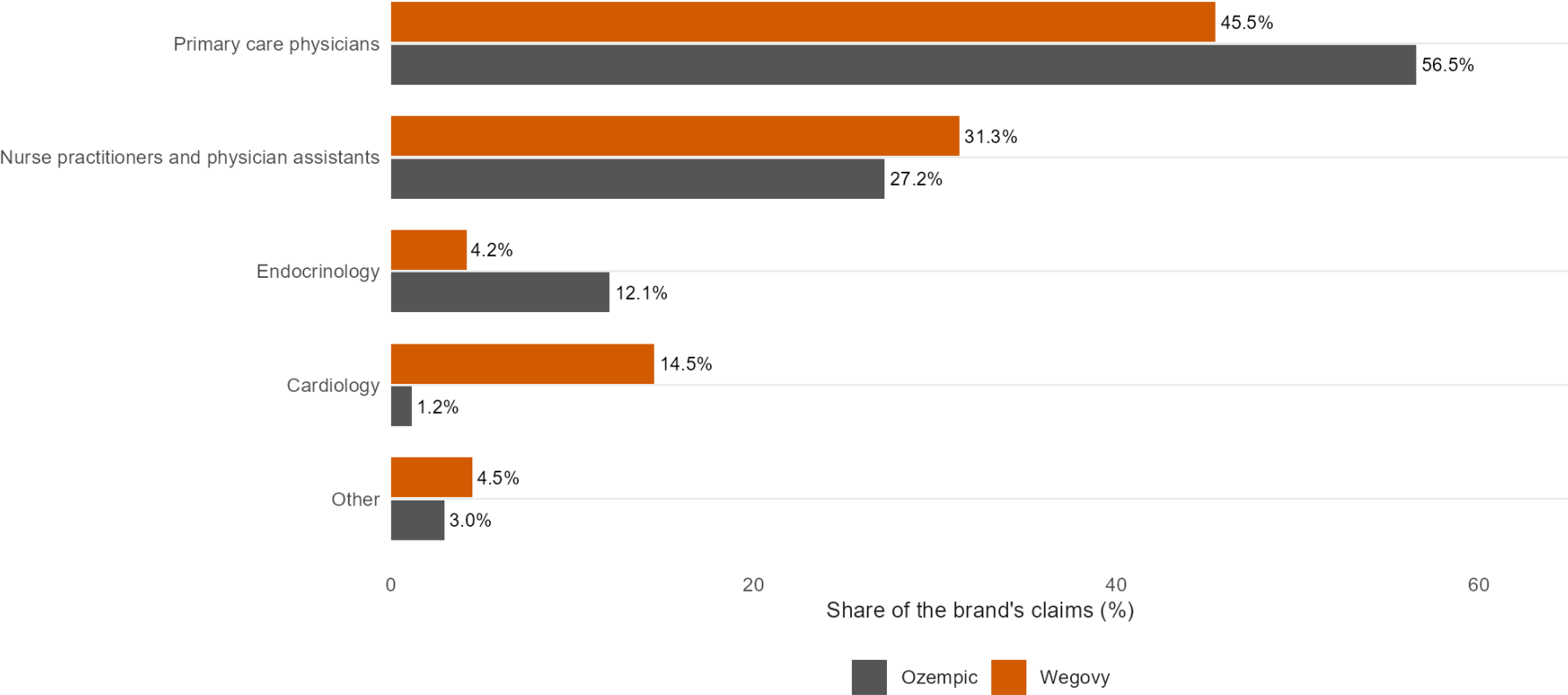


Source: CMS Medicare Part D Prescribers by Provider and Drug (rows with at least 11 claims), NPPES. Part D reflects diabetes and other covered uses, not obesity-brand adoption; counts are claims, and only prescribers with 11 or more claims for a drug appear.

Source: CMS Medicare Part D Prescribers. Part D reflects diabetes and other covered uses, not obesity-brand adoption.

# Prescribers: cardiology wrote 14.5% of Wegovy's Part D claims against 1.2% of Ozempic's, after the cardiovascular indication

Share of 2024 Part D claims by prescriber specialty group, Wegovy (cardiovascular indication since 2024-03-08) versus Ozempic. One year of data: descriptive only.

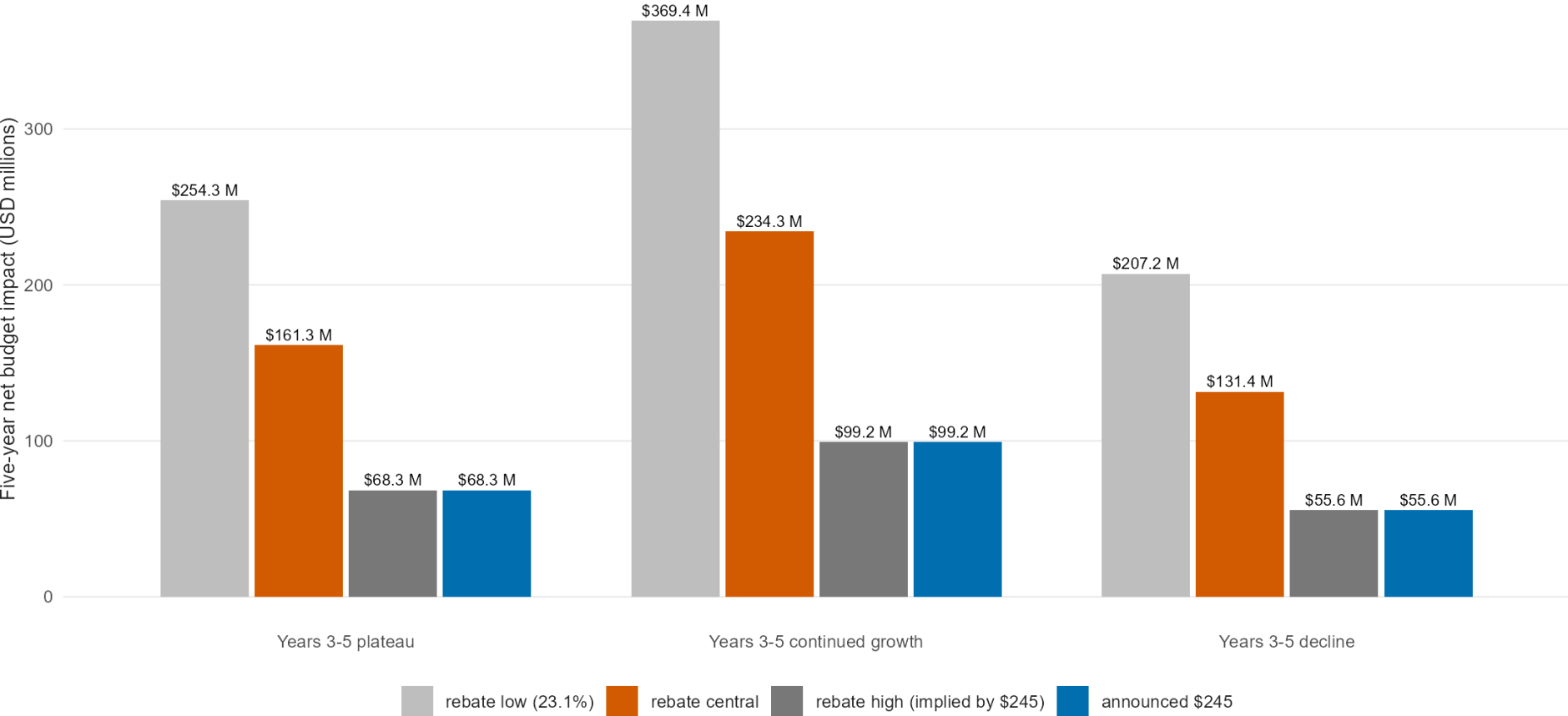


Source: CMS Medicare Part D Prescribers by Provider and Drug (rows with at least 11 claims), NPPES. Part D reflects diabetes and other covered uses, not obesity-brand adoption; counts are claims, and only prescribers with 11 or more claims for a drug appear.

Source: CMS Medicare Part D Prescribers, 2024 (76,176 Wegovy claims from 3,956 prescribers). Part D reflects diabetes and other covered uses, not obesity-brand adoption.

# Value: five-year net cost ranges from \$55.6 million to \$369.4 million for 1,000,000 enrollees; the central case is \$161.3 million

Net cost by years 3-5 scenario and by rebate or price assumption (prior authorization as observed, multiplier 1.0; incremental to no coverage).

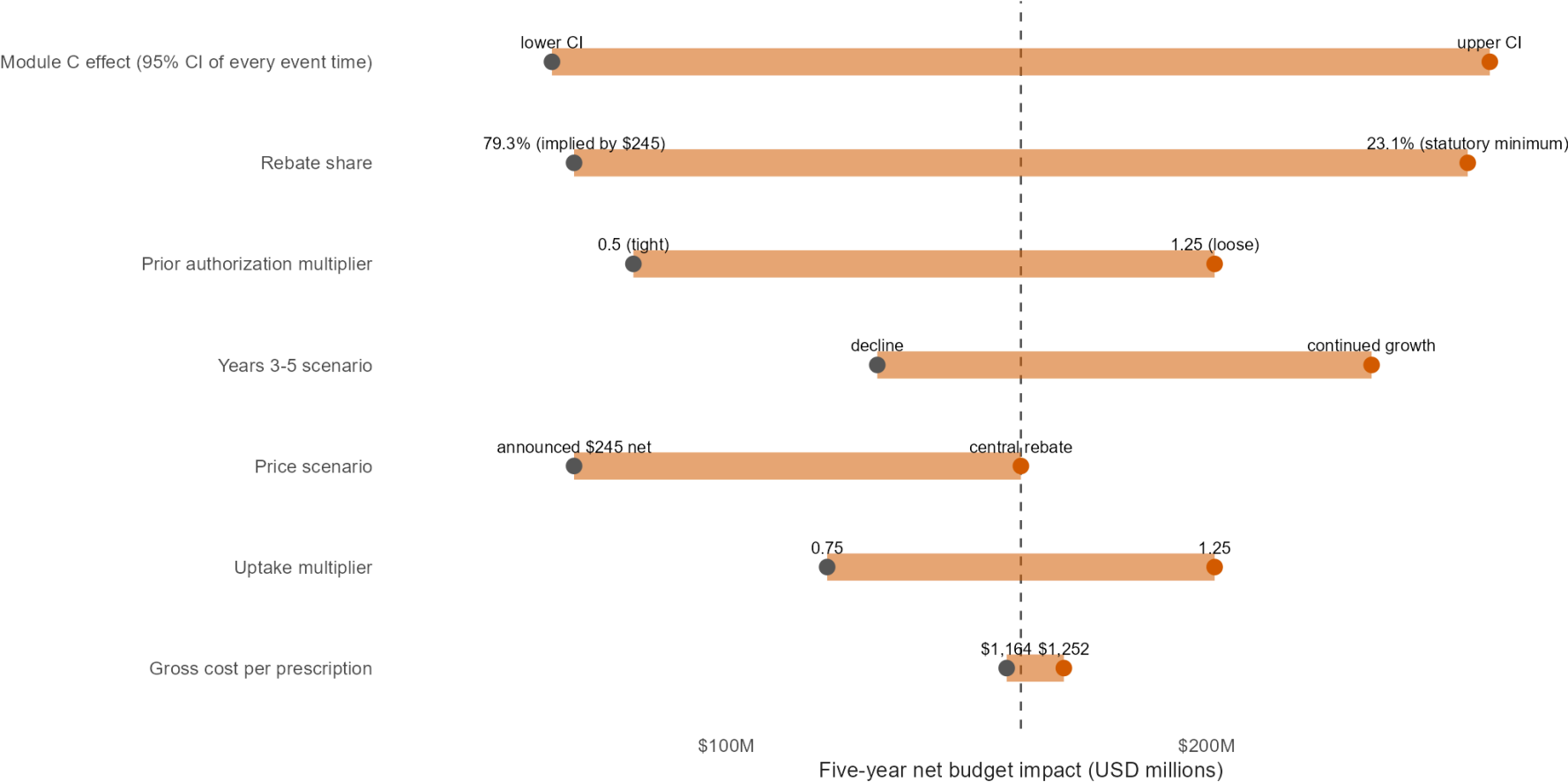


Source: Module C estimates (CMS State Drug Utilization Data, Medicaid enrollment); SDUD gross reimbursement; statutory rebate floor 42 U.S.C. 1396r-8; White House fact sheet Nov 2025. SDUD amounts are gross of rebates; rebates, prior authorization and years 3-5 are assumptions.

Source: Module C effects, SDUD gross reimbursement, statutory rebate floor (42 U.S.C. 1396r-8), White House fact sheet. Rebates, prior authorization and years 3-5 are assumptions; no medical cost offsets.

# Value: the rebate and the uncertainty in the coverage effect move the answer most

One-way sensitivity: each parameter moved across its range with the others at central values (incremental to no coverage; central = plateau, PA as observed, rebate 51.2% = midpoint of 23.1% and 79.3%).

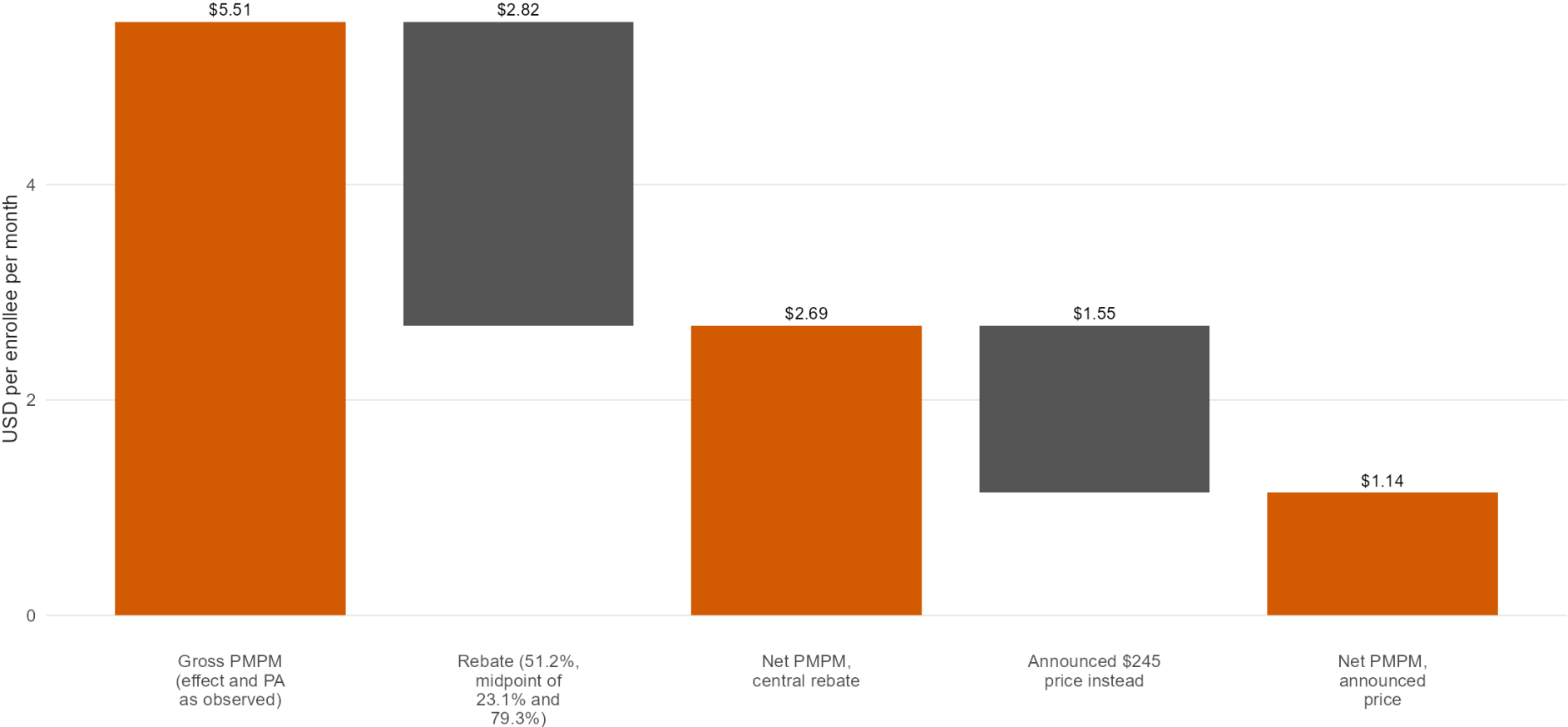


Source: Module C estimates (CMS State Drug Utilization Data, Medicaid enrollment); SDUD gross reimbursement; statutory rebate floor 42 U.S.C. 1396r-8; White House fact sheet Nov 2025. SDUD amounts are gross of rebates; rebates, prior authorization and years 3-5 are assumptions.

Source: Module E one-way sensitivity. Central rebate 51.2% is the midpoint of 23.1% (statutory minimum) and 79.3% (implied by the \$245 price); actual net prices are confidential.

# Value: per enrollee per month, a gross \$5.51 becomes \$2.69 net at the central rebate and \$1.14 at the announced price

Waterfall from gross PMPM (effect and prior authorization as observed in the covering states) to net PMPM. The rebate step is an assumption; the \$245 price is an announced programme price.



Source: Module C estimates (CMS State Drug Utilization Data, Medicaid enrollment); SDUD gross reimbursement; statutory rebate floor 42 U.S.C. 1396r-8; White House fact sheet Nov 2025. SDUD amounts are gross of rebates; rebates, prior authorization and years 3-5 are assumptions.

Source: Module E. The rebate step is an assumption; the announced price is a programme price, not a confirmed Medicaid net price.

# Recommendations: considerations for a brand or access lead, from the evidence above

| Finding                                                                                                       | So what                                                                   | What a brand or access lead might consider                                                                                  |
|---------------------------------------------------------------------------------------------------------------|---------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------|
| Coverage added 12.2 prescriptions per 1,000 enrollees per quarter and grew over eight quarters                | Uptake builds over about two years rather than at launch                  | Planning access support over a multi-year ramp instead of a launch spike                                                    |
| Coverage runs through managed care in South Carolina and Rhode Island                                         | Decisions sit with managed-care organisations as well as state programmes | Evidence and criteria materials aimed at managed-care medical and pharmacy directors (to be tested in the research pack)    |
| Coverage ended in California and Pennsylvania (-67.2% and -90.8%, preliminary)                                | Budget predictability decides whether coverage lasts                      | Showing a clear range of cost scenarios: \$2.69 per enrollee per month at the central rebate, \$1.14 at the announced price |
| Primary care and nurse practitioners and physician assistants wrote 55.8% and 27.7% of Part D incretin claims | The prescriber base is broad, not specialist                              | Weighing primary-care reach (Part D shows diabetes and other covered uses; confirm for obesity with other data)             |
| The rebate is unknown and drives the range (\$55.6 to \$369.4 million)                                        | Public data cannot settle net cost                                        | Treating the budget range as a conversation starter and checking it against a payer's own net price                         |

# Appendix: data, methods and limits

- **Data.** CMS Medicaid State Drug Utilization Data and enrollment, Medicare Part D Prescribers, NADAC, NHANES, MEPS, KFF polls, FDA labels and state Medicaid documents; all public aggregate data. 54 dbt models, 126 tests.
- **Methods.** Callaway and Sant'Anna difference-in-differences (staggered adoption) with imputation of suppressed cells; survey-weighted NHANES estimates; ISPOR-style budget impact model with a probabilistic sensitivity analysis.
- **Limits.** Gross, not net, of rebates; ten covering states; eligibility is a lower bound; Part D is not obesity use; scenarios, not forecasts.
- **Links.** Report ([report/report.html](#)), plans and deviations ([analysis/outputs/deviations.md](#)), key numbers ([analysis/outputs/key\\_numbers.csv](#)), repository [github.com/erickyegon/incretin-access-value](#).
- No company affiliation or endorsement. I used AI tools to help write code and documentation. The study design, methods and conclusions are my own, and I verified all results.